Statement of Deficiency Report

Department of Health
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Newport Academy, 3804 Hastings Ave, W.
Port Townsend, WA 98368-9642
Agency Name and Address

Angeleena May
Administrator

Investigation
Inspection Type

Tuesday, June 16, 2020
Investigation Start Date

MCB33
Investigator Number

2020-705
Case Number

RTF.FS.60957241
License Number

Mental Health
BHA/RTF Agency Services Type

Please note that the deficiencies/violations/observations noted in this report are not all-inclusive, but rather were deficiencies/violations/observations that were observed or discovered during the investigation.

Deficiency Number and Rule Reference	Findings	Plan of Correction
246-337-080 Resident care services. (3)(b) RTF's performing any duties described in (a) of this subsection must meet the following staffing requirements: (i) A registered nurse, licensed practical nurse, or prescriber must be available on-site during medication administration or while restraint or seclusion is being used, and otherwise available by phone twenty-four hours per day, seven days per week.	Based on interviews and policy and procedure review, the facility failed to implement policies and procedures that required a registered nurse, licensed practical nurse, or prescriber to be on-site during medication administration. Failure to implement policies and procedures that require a registered nurse, licensed practical nurse, or prescriber to be on-site during medication administration and	See attached pages to cover the deficiencies in this section and the corrective action plan. Plan of correction was implemented by 10/1/2021 and is monitored by Nurse Manager by completing monthly audits conducted by Nurse

	improper administration by an unlicensed personnel places patient at risk for harm Findings included: 1. Review of the facilities policy and procedure titled, "Health Care Services", number R18 dated 07/30/19 states the facility will provide a registered nurse, licensed practical nurse, or prescriber be available on-site during medication administration. 2. During an interview on 06/09/21, with Staff E, Registered Nurse, Nursing Manager, stated that the facility should have a nurse administer the medication to the patient and not the Care Coordinators but they don't have enough nurses to cover all the shifts. Staff E stated that they didn't want the patients to be allowed to handle their own medication for self-administration. 3. During interviews on 06/09/21, Staff L, Counselor, Staff N, Care Coordinator 3, Lead, and Staff O, Care Coordinator, stated that when there is no nurse on duty, the care coordinators will give out medication to the patients. Staff L, N, and O stated that they take the medication out of the bottle and put it in a dispensing cup and hand it to the patient while they observe.	Manager, one CC will be pulled aside at random and quizzed as to proper med observation procedures. This will be documented on monthly audit sheet that is submitted to compliance.
WAC 246-337-105 Medication management.	Based on interviews and policy and procedure review the facility failed to implement policies	See attached pages to cover the deficiencies in this

The licensee is responsible for implementing policies and procedures for the control and appropriate use of all drugs within the RTF in accordance with all applicable state and federal regulations. The policies and procedures to implement this number must be developed, approved, and reviewed by a health care prescriber and the RTF administrator, and must be consistent with this chapter. (4) Prescribing and administering drugs. (f) The licensee shall address the way(s) medications are administered including: (i) Staff-administered medication in which licensed staff operating within their scope of practice remove the drug from the container and provide it to the resident for ingestion or otherwise administer the drug to the resident; (ii) Observed self-administration of medication in which residents obtain their container of medication from a supervised and secure storage area, remove the dose needed, ingest or otherwise take the medication as directed on the label while being observed by staff.	and procedures that required a licensed staff working within their scope of practice to administer medications to patients. Failure to implement policies and procedures that require a licensed staff working within their scope to remove the drug from the container and provide it to the patient for ingestion puts patient at risk for medication errors and risk of harm. Findings included: 1. Review of the facilities policy and procedure titled, "Medication Administration", number R21 dated 07/30/19, showed the following: a. That staff-administered medication will be performed by licensed staff working within their scope of practice. Licensed staff will remove the drug from the container and provide it to the patient for ingestion. b. Self-administered drugs are obtained by the patient who will remove the drug from the container by its proper dose and while being observed, will ingest the medication. 2. During an interview with Staff E, Registered Nurse Manager, on 06/09/21, Staff E stated the facility should have a nurse administer the medication to the patient and not the Care Coordinators but they don't have enough	section and the corrective action plan. Plan of correction was implemented by 10/1/2021 and is monitored by Nurse Manager by completing monthly audits conducted by Nurse Manager, one CC will be pulled aside at random and quizzed as to proper med observation procedures. This will be documented on monthly audit sheet that is submitted to compliance.

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Introduction

We require that you submit a plan of correction for each deficiency listed on the statement of deficiency form. Your plan of correction must be Submitted to DOH within fourteen calendar days of receipt of the list of deficiencies.

You are required to respond to the statement of deficiencies by submitting a plan of correction (POC). Be sure to refer to the deficiency number. If you include exhibits, identify them and refer to them as such in your POC.

Descriptive Content

Your plan of correction must provide a step-by-step description of the methods to correct each deficient practice to prevent recurrence and provide information that ensures the intent of the regulation is met.

An acceptable plan of correction must contain the following elements:

- The plan of correcting the specific deficiency;
- The procedure for implementing the acceptable plan of correction for the specific deficiency cited;
- The monitoring procedure to ensure that the plan of correction is effective and that specific deficiency cited remains corrected and/or in compliance with the regulatory requirements;
- The title of the person responsible for implementing the acceptable plan of correction.

Simply stating that a deficiency has been "corrected" is not acceptable. If a deficiency has already been corrected, the plan of correction must include the following:

- How the deficiency was corrected,
- The completion date (date the correction was accomplished),
- How the plan of correction will prevent possible recurrence of the deficiency.

Completion Dates

The POC must include a completion date that is realistic and coinciding with the amount of time your facility will need to correct the deficiency. Direct care issues must be corrected immediately and monitored appropriately. Some deficiencies may require a staged plan to accomplish total correction. Deficiencies that require bids, remodeling, replacement of equipment, etc., may need more time to accomplish correction; the target completion date, however, should be within a reasonable and mutually agreeable time-frame.

Continued Monitoring

Each plan of correction must indicate the appropriate person, either by position or title, who will be responsible for monitoring the correction of the deficiency to prevent recurrence.

Checklist:

- Before submitting your plan of correction, please use the checklist below to prevent delays.
- Have you provided a plan of correction for each deficiency listed?
- Does each plan of correction show a completion date of when the deficiency will be corrected?
- Is each plan descriptive as to how the correction will be accomplished?
- Have you indicated what staff position will monitor the correction of each deficiency?
- If you included any attachments, have they been identified with the corresponding deficiency number or identified with the page number to which they are associated?

Your plan of correction will be returned to you for proper completion if not filled out according to these guidelines.

Note: Failure to submit an acceptable plan of correction may result in enforcement action.

Approval of POC

Your submitted POC will be reviewed for adequacy by DOH. If your POC does not adequately address the deficiencies, you will be sent a letter detailing why your POC was not accepted.

Questions?

Please review the cited regulation first. If you need clarification or have questions about deficiencies, you must contact the investigator who conducted the investigation.

Medication Observation CAP

In response to Citation in reference to: WAC 246-337-080 Resident care services.

At Newport Academy Port Townsend, if a Nurse is not available for a med pass shift, CC staff will participate in medication observation. The RN nurse manager is available by phone 24 hours a day, 7 days a week at this location if any issues arise on site.

In response to Citation in reference of: WAC 246-337-105 Medication management.

In the absence of Nursing staffing during medication pass, trained CC staff will do medication observation.

When conducting medication observation, CC staff will observe the following protocol:

Observed medication self-administration protocol

Protocol to be followed at facilities in Washington State when a licensed medical professional is not available to give medications.

1) Open medication administration in Lightning Step
2) Client arrives at medication administration station
3) Open client's MAR (medication administration record)
4) Determine which medications are due to be administered
5) Compare ordered medications to what is available in client's medication box, compare orders in the MAR to pharmacy labels
6) Remove entire bottle or bubble pack of intended medication from the box
7) Confirm the 5 Rights of Medication Administration (Client, Medication, Dose, Route, Time)
8) Hand one medication bottle or bubble pack to client at a time
9) Instruct client to remove intended number of pills from bottle or bubble pack and place in medication cup
10) Client returns the bottle or bubble pack before the next medication is provided for them to dispense
11) Repeat steps 8-10 as many times as needed until completed
12) Document in the Lightning Step MAR that the dose was administered, write a note in the drop-down notes section stating "observed self-administration"

If client asks for a PRN medication, follow the same steps. A client should be able to ask for their PRN medication by name.

In order to participate in medication observation, CC's must complete the following training:

1. Medication Observation basics presentation with Nurse Manager.
2. Check off documentation filled out by CC and Nursing staff to include:
   a. Med room orientation
   b. Mock med observation of regular, controlled and PRN meds
   c. Three days of observing, participating, and then running a med pass via observation under supervision of a nurse

Documentation of completed med observation training signed off by appropriate staff will be kept in a folder in the nursing office filing cabinet.

To document ongoing compliance with this, during monthly audit conducted by Nurse Manager, one CC will be pulled aside at random and quizzed as to proper med observation procedures. This will be documented on monthly audit sheet that is submitted to compliance.

This CAP will be fully enacted by 10/1/21, with the responsibility to do so falling to the Nursing Manager of the Port Townsend Site.

STATE OF WASHINGTON

DEPARTMENT OF HEALTH

September 17, 2021

Re: Case Number: 2020-705
License Number: RTF.FS.60957241
Acceptable Plan

Dear: Angeleena May, LMHC, Clinical Director

This letter is to inform you that after careful review of the Plan of Correction (POC) you submitted for the investigation recently conducted at your agency, the Department has determined that the POC is acceptable. You stated in your plan that you will implement corrective actions by the specified timeline. By this, the Department is accepting your Plan of Correction as your confirmation of compliance.

Based on the scope and severity of the deficiencies listed in your statement of deficiency report, the Department will not conduct an unannounced follow-up compliance visit to verify that all deficiencies have been corrected.

The Department reserves the right to pursue enforcement action for any repeat and/or uncorrected deficiencies based on applicable statute and rules.

Investigator: MCB33
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HSQA/Office of Health Systems Oversight
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